

Medical Legal Document Analysis Report

Extracted Variables

Agreed Upon Body Parts: RIGHT SHOULDER BLADE, RIGHT SCAPULA, right shoulder, right elbow

Disagreed Upon Body Parts: neck, right trapezius

Previous Injuries: RIGHT SHOULDER JOINT PAIN, CERVICAL RADICULOPATHY, TENDINITIS OF RIGHT SHOULDER, IMPINGEMENT SYNDROME OF LEFT SHOULDER, CERVICAL MYOFASCIAL PAIN SYNDROME, LUMBAR RADICULOPATHY, CHRONIC LOW BACK PAIN, Hx of orbital fracture, facial surgery and metal plate

Applicant Attorney Alleged Body Parts: RIGHT SHOULDER BLADE, RIGHT SCAPULA, right shoulder, right elbow, neck, right trapezius

1. Demographics

Q1: Look at the defense attorney, applicant attorney letter and insurance company and itemize all the names and addresses of them.

A: Itemized parties and addresses (as shown in the records) - Defense Attorney - Morgan and Leahy LLP - 30101 Agoura Court Suite 200, Agoura Hills, CA 91301 - Applicant Attorney (Requesting Party) - Emily Mehr, Esq., MEHR ASSOCIATES IRVINE - 17500 Red Hill Ave Suite 210, Irvine, CA 92614 - Insurance Company / Claims Administrator - SEDGWICK CMS - PO BOX 14188, Lexington, KY 40512

Q2: Look throughout the document and fill in the following, note that most information but sometimes not all can be found in notes, usually applicant or defense attorney letters: 1. Claimant Name: (Last, First) 2. Claimant Date of Birth: (mm/dd/yyyy) 3. Claimant Employer: 4. Claimant Occupation: 5. Claimant Date of Injury: (mm/dd/yyyy) 6. Claim number: 7. WCAB Number: 8. Date of Exam: (Leave this empty)

A: 1. Claimant Name: Tiu, Michelle 2. Claimant Date of Birth: 10/31/1976 3. Claimant Employer: Kaiser Permanente 4. Claimant Occupation: Not specified 5. Claimant Date of Injury: 05/28/2024 6. Claim number: 2024050001029 7. WCAB Number: ADJ19391023 8. Date of Exam:

2. History Of Injury (Patients Perspective)

Q1: Review the patients entire history of injury as described from the patients perspective and summarize it in a single paragraph.

A: According to the patient, on 05/28/2024 she developed right-sided neck and shoulder/scapular pain while working in a non-ergonomic exam room and office, performing constant keyboarding with her arms outstretched and frequent overhead reaching for equipment, which led to right shoulder and elbow complaints; she described right-sided neck myofascial pain radiating into the right upper trapezius down to the elbow and the 4th–5th fingers, and noted that when she lies on her right arm her entire right arm goes numb, with pain aggravated by any movement, using her arms overhead, and pushing/pulling, and alleviated by acupuncture and ibuprofen (later reporting moderate improvement with acupuncture/myofascial treatment, better range of motion, ability to lie down without severe neck pain, waking without pain in the morning, but persistent discomfort and pain with reaching out or above chest level); she completed 6/6 physical therapy sessions by 07/29/2024 and, while on modified duty (acting as a greeter), felt capable of returning to regular tasks if allowed brief stretch breaks morning

and afternoon, and on 07/15/2024 asked to be released to full duty, expressing upset that her work restrictions reduced her lunch to 30 minutes and increased booking responsibilities; accepted/treated body parts include RIGHT SHOULDER BLADE/RIGHT SCAPULA with diagnoses of RIGHT SCAPULALGIA, RIGHT TRAPEZIUS STRAIN (subsequent encounter), and NECK MUSCLE STRAIN (subsequent encounter), and a 06/26/2024 cervical spine X-ray referenced loss/straightening of cervical lordosis with minimal degenerative changes (most at C6-7 and C7-T1), while a 08/28/2024 nerve conduction/EMG exam history reiterated her symptoms and confirmed (from testing context) right-sided sensory symptoms consistent with her reports.

3. History Of Injury (Physicians Perspective)

Q1: Review the patients entire history of injury as described from the physicans notes and summarize it in a single paragraph

A: On 05/28/2024, while working for KP SCAL, the patient developed right-sided neck/shoulder pain attributed to a non-ergonomic exam room and office with constant keyboarding with arms outstretched and frequent overhead reaching for equipment; accepted body parts were documented as RIGHT SHOULDER BLADE/RIGHT SCAPULA (later also listed as right shoulder and right elbow), Claim No. 2024050001029. Initial Occupational Health assessment on 07/11/2024 recorded cervical spine strain, trapezius strain, and right arm paresthesia (rule out radiculopathy/peripheral neuropathy), with cervical spine X-ray showing “lower level mild degenerative changes with loss of cervical lordosis”; dedicated XR on 06/26/2024 demonstrated straightening of cervical lordosis (positional), minimal degenerative changes with disc space narrowing and osteophyte formation most severe at C6–7 and C7–T1, unchanged from prior, and no acute fracture or soft-tissue abnormality (finalized 06/27/2024). By 07/29/2024 she reported moderate improvement with acupuncture and myofascial treatment, Tylenol and anti-inflammatories; pain remained intermittent 3–4/10, aggravated by reaching out or above chest level, overhead use, and pushing/pulling; exam findings from the prior visit included protracted neck posture, bilateral upper trapezius/levator scapulae tenderness (L>R) with a right upper trapezius trigger point and Spurling’s reproducing right trapezius pain. Management included ibuprofen 400 mg three times daily as needed, continuation of Physical Therapy for home program and manual therapy, ongoing myofascial release, completion of ordered nerve conduction studies/electromyography, an ergonomic evaluation request, consideration of trigger point injection, and possible repeat/advanced imaging if symptoms persisted. Work status fluctuated from a patient-requested release to full duty on 07/15/2024 to usual and customary work with recommended stretch breaks (10–20 minutes on 07/11/2024; then 20-minute morning and afternoon breaks from 07/31/2024 through 09/05/2024, extended to 10/22/2024 due to clinic cancellation). On 08/28/2024, EMG/NCS for right-sided neck myofascial pain radiating into the right upper trapezius down to the elbow and 4th–5th fingers (with positional right arm numbness when lying on the right arm) showed prolonged right median sensory latency across the wrist with otherwise normal median motor/ulnar/radial studies and needle EMG demonstrating enlarged/unstable motor unit potentials with reduced recruitment in the right C8 myotome without active denervation—conclusions: mild right carpal tunnel syndrome (median nerve entrapment at the wrist) and chronic right C8 radiculopathy. Active diagnoses carried forward include RIGHT SCAPULALGIA (M89.8X1), RIGHT TRAPEZIUS STRAIN, SUBSEQ (S46.811D), and NECK MUSCLE STRAIN, SUBSEQ (S16.1XXD).

4. Chief Complaints

Q1: Describe the patients chief complaints for all the agreed upon body parts

A: Below is a summary of the patient’s chief complaints for the accepted/agreed body parts in the workers’ compensation claim. Accepted body parts documented in the record include: - “RIGHT SHOULDER BLADE, RIGHT SCAPULA” (noted 07/15/2024) - “right shoulder and right elbow” (noted 08/28/2024) Right shoulder/shoulder blade/scapula - Chief Complaint listed at visit: “SHOULDER

INJURY Right DOI - 5/28/2024.” (07/29/2024 Office Visit) - Symptom description: “right sided neck myofascial pain that radiates into the right upper trapezius down to the elbow and 4th-5th finger. When lays on right arm she feels her right entire arm will go numb.” (08/28/2024 EMG/NCV report) - Pain quality/location and aggravators: “bilateral neck and shoulder girdle pain that is aching and numbing... Pain still aggravated with reaching out or above chest level with the arms... aggravated by any movement, using arms overhead, and pushing and pulling.” (07/29/2024 Office Visit – Subjective) Right elbow - Chief Complaint listed at visit: “ELBOW INJURY Right DOI - 5/28/2024.” (07/29/2024 Office Visit) - Symptom description involving the elbow: pain “radiates into the right upper trapezius down to the elbow and 4th-5th finger,” and “When lays on right arm she feels her right entire arm will go numb.” (08/28/2024 EMG/NCV report) Additional symptom noted relevant to the right upper extremity - “Right arm paresthesia – rule out radiculopathy, peripheral neuropathy.” (07/11/2024 Office Visit Plan)

Q2: Describe the patients chief complaints for all the disagreed upon body parts

A: Summary of “disagreed upon” body parts and the patient’s chief complaints Context on accepted body parts (for contrast): - 07/15/2024 (Telephone, OHS): “Accepted body part(s): RIGHT SHOULDER BLADE, RIGHT SCAPULA.” - 08/28/2024 (Telephone, OHS): “Accepted body part(s): right shoulder and right elbow.” Accordingly, body parts outside the accepted list (i.e., in dispute) are the cervical spine/neck region (including right trapezius) and right upper-extremity neurologic components (radicular symptoms/right arm/hand; carpal tunnel/right wrist). Chief complaints for the disagreed-upon body parts (verbatim from records where available) 1) Cervical spine/neck (including right trapezius/neck myofascial pain) - “bilateral neck and shoulder girdle pain that is aching and numbing. Pain is rated at 3-4/10 ... intermittent” (07/29/2024 OHS progress note) - “Pain still aggravated with reaching out or above chest level with the arms.” (07/29/2024 OHS progress note) - Aggravating factors: “any movement, using arms overhead, and pushing and pulling” (07/29/2024 OHS progress note) - Alleviating factors: “acupuncture, ibuprofen” (07/29/2024 OHS progress note) - Persistent focal complaint: “Massage/myofascial release has been exceptionally helpful in reducing the tight ‘knot’ on the right side of her neck, but discomfort persists” (07/29/2024 OHS progress note) - Earlier assessment capturing the patient’s symptom report: “Right arm paresthesia- rule out radiculopathy, peripheral neuropathy” (07/11/2024 OHS note) 2) Right trapezius (part of the cervical/neck myofascial complex) - “right sided neck myofascial pain that radiates into the right upper trapezius down to the elbow and 4th-5th finger” (EMG/NCV visit, 08/28/2024, Patient History/Notes) 3) Right upper extremity neurologic complaints (radiculopathy/right arm/hand; carpal tunnel/right wrist) - Radiation/paresthesia: “radiates into the right upper trapezius down to the elbow and 4th-5th finger” (08/28/2024 EMG/NCV Patient History/Notes) - Positional numbness: “When lays on right arm she feels her right entire arm will go numb.” (08/28/2024 EMG/NCV Patient History/Notes) - Sensory complaint noted in assessment: “Right arm paresthesia” (07/11/2024 OHS note)

5. Claimants Job

Q1: Describe the job description of the claimants job

A: Based on the records, the claimant (Michelle Tiu) is employed by Kaiser Permanente Southern California (“Employer: KP SCAL”) and performs patient-care/clinic-based duties in an exam room/office setting with the following documented job activities and physical demands: - Work setting: “non-ergonomic exam room and office.” (Mechanism of injury, 07/29/2024 Occupational Health note) - Repetitive computer work: “constant keyboarding with arms outstretched.” (07/29/2024 Occupational Health note) - Reaching/overhead tasks: “frequent over head reaching for equipment.” (07/29/2024 Occupational Health note) - On modified duty: served as a “greeter” (patient stated she was “work modified, acting as greeter”). (07/29/2024 Occupational Health note) - Patient flow/administrative concerns noted during restrictions: patient complained restrictions “reduced her lunch to 30 min and having to rook her own pts, and see more patient?” (07/15/2024 telephone note by N. Pham■Bailey, MD) Employer identification: - Employer: KP SCAL (Kaiser Permanente Southern California). - Claim Number: 2024050001029. - Accepted body parts related to work activities: “RIGHT SHOULDER

BLADE, RIGHT SCAPULA" (07/15/2024 telephone note).

Q2: Describe the specific job duties of the claimants job

A: Below are the specific job duties described in the records for the claimant, Michelle Tiu (Employer: KP SCAL), using the exact language where available: - Reaching out and above shoulder level frequently in a "non-ergonomic exam room and office." (Mechanism of injury, Occupational Health Services (OHS) note, 07/29/2024) - "Constant keyboarding with arms outstretched." (OHS note, 07/29/2024) - "Frequent over head reaching for equipment." (OHS note, 07/29/2024) - Modified duty assignment: "acting as greeter." (OHS note, 07/29/2024) - Patient-reported workload/tasks: she was "upset that her work restrictions somehow reduced her lunch to 30 min and having to rook her own pts, and see more patient?" (Telephone note, 07/15/2024)

6. Work Status

Q1: Find the most recent work status and any work restrictions the claimant has

A: Most recent work status and restrictions - Work status: Modified activity at work and at home from 07/31/2024 through 10/22/2024 (extended due to clinic cancellation on 08/28/2024). - Source language: "This patient is placed on modified activity at work and at home from 7/31/2024 through 9/5/2024... Needs and/or Restrictions Recommend 20 minute stretch break in morning and 20 minute stretch break in afternoon." Extension noted: "ActivityRx 7/31/24-10/22/2024 pended... ARX dates extended due to clinic cancellation... updated work note is available on kp.org." (08/28/2024 Occupational Health Services telephone encounter) - Specific restrictions: - "Recommend 20 minute stretch break in morning and 20 minute stretch break in afternoon." - Contingency: "If modified activity is not accommodated by the employer then this patient is considered temporarily and totally disabled from their regular work for the designated time and a separate off work order is not required." Note: Although a line on the Industrial Work Status Report states, "The patient was evaluated and deemed able to return to work at full capacity on 7/29/2024," the operative and most recent status is the modified activity with the above stretch-break restrictions, extended through 10/22/2024.

Q2: Has the patient been taken off work? (If true write "Patient is still working" if false write Patient stopped working on (date) + write a 2 sentence explanation)

A: Patient is still working

7. Previous Injuries

Q1: Has the patient described any previous injuries If no then write "Patient has has to previous injuries" If yes then for each of the previous injuries reiterate how the patient described the injuries in a few sentences.

A: Patient has has to previous injuries

8. Medical History

Q1: Write a list of the entire non surgical medical history of the patient if applicable, if not, write "No past medical history"

A: - Chronic low back pain [G89.29, M54.5] (CT lumbar spine 05/13/2020: diffuse mild posterior disc bulges at L3-4, L4-5, L5-S1; no significant spinal canal or neuroforaminal stenosis) - Lumbar radiculopathy [M54.16] - Cervical radiculopathy [M54.12]; EMG/NCS 08/28/2024: chronic right C8 radiculopathy (chronic denervation in right C8 myotome, no ongoing denervation) - Mild right carpal tunnel syndrome (median nerve entrapment at the wrist) per EMG/NCS 08/28/2024 - Cervical myofascial pain syndrome - Cervical spine degenerative changes: minimal/mild disc space narrowing

and osteophyte formation, most severe at C6-7 and C7-T1; straightening/loss of cervical lordosis (cervical spine X-rays 06/09/2023 and 06/26/2024) - Neck muscle strain, subsequent encounter [S16.1XXD] - Right trapezius strain, subsequent encounter [S46.811D] - Right scapalgia [M89.8X1] - Tendinitis of right shoulder - Impingement syndrome of left shoulder - Presbyopia [H52.4] - Bilateral myopia [H52.13] - Bilateral astigmatism [H52.203] - Mammography finding: both breasts are extremely dense; BI-RADS 1 (negative) screening mammogram on 03/18/2023; routine screening recommended within 2 years - Family history of breast cancer - Family history of ovarian cancer - Family history of glaucoma (both parents) - Hx of orbital fracture (sic)

9. Surgical History

Q1: Write a list of the entire surgical history of the patient if applicable, if not, write "No past surgical history"

A: - Facial surgery with metal plate (date not specified). Documented as the reason the patient could not undergo MRI.

10. Allergies

Q1: Write a list of all the patient's allergies if applicable, if not, write "Patient has no provided allergies"

A: Patient has no provided allergies

11. Current Medications

Q1: Write a list of all the medications the claimant is currently taking

A: Based on the records provided, there are no outpatient medications documented as "currently taking." - Note: At the 07/29/2024 Occupational Health Services visit, the chart states: "No outpatient medications have been marked as taking for the 7/29/24 encounter." The patient reported improvement with "Tylenol and anti-inflammatories," and ibuprofen 400 mg three times daily as needed was recommended, but no active medication list was recorded as being taken.

12. Social History

Q1: Write a list of any alcohol recreational drug, recreational tobacco, or vape use the patient has had, as well as when they did it according to the medical record.

A: No documented use found in the provided records. - Alcohol: Not documented. - Recreational drug use: Not documented. - Tobacco use (including vaping): Not documented. Note: Across the encounters and reports provided (e.g., ED visit 11/06/2016; imaging reports 2017–2023; Occupational Health Services visits and procedures 07/2024–08/2024; Optometry 08/26/2024), no social history entries specifying alcohol, recreational drug, tobacco, or vape use were recorded.

13. Family History Of Illness

Q1: Write a list of any past medical or surgical history of family members

A: Family history documented in the records: - FHx OF BREAST CANCER - FHx OF OVARIAN CANCER - FHx of Glaucoma both parents. Note: No family surgical history is documented in the provided records.

16. Diagnostic Reports

Q1: Summarize each and every MRI with the diagnostic results of the MRI, in 5 or less sentences in chronological order. If there are no studies/results output "none of these studies were completed" with a brief explanation

A: none of these studies were completed — The record contains no MRI orders or MRI results. Documented imaging consists of a CT lumbar spine without contrast on 05/13/2020 (performed because the patient was "unable to get MRI due to facial surgery and metal plate"), screening mammography on 03/18/2023, and multiple radiographs of the cervical spine and right shoulder in 2023–2024. An EMG/NCS was performed on 08/28/2024. Therefore, there are no MRI diagnostic results to summarize.

Q2: Summarize each and every CT or CAT scan with the diagnostic results of the CT or CAT scan, in 5 or less sentences in chronological order.

A: - 05/13/2020 — CT Lumbar Spine without contrast: Normal lumbar lordosis with no compression fracture, spondylolisthesis, or significant disc space narrowing. Mild posterior disc bulges at L3-4 (2 mm), L4-5 (3 mm), and L5-S1 (3 mm) with mild ligamentum flavum hypertrophy at L3-4 and L4-5. No significant spinal canal stenosis or neuroforaminal stenosis; paraspinal soft tissues intact. Impression: No acute findings; diffuse mild posterior disc bulges at L3-4, L4-5, and L5-S1 without significant canal or foraminal stenosis.

Q3: Summarize each and every X-Rays with the diagnostic results of the X-Rays, in 5 or less sentences in chronological order.

A: - 03/18/2023 – Bilateral digital screening mammogram: Both breasts are extremely dense, which lowers the sensitivity of mammography. No significant masses, calcifications, or other findings are seen in either breast. There has been no significant interval change. Impression: Negative (BI-RADS 1). A routine screening mammogram is recommended within 2 years. - 06/07/2023 – XR Right Shoulder (2 or more views): Osseous mineralization is normal. No acute fracture is seen. The acromioclavicular and glenohumeral joint spaces are maintained. Regional soft tissues appear normal. - 06/09/2023 – XR Cervical Spine (AP, lateral, and odontoid): Cervical vertebral bodies are normal in height; the alignment is normal. No fracture is identified and no compression fracture. Mild degree of disc space narrowing at C6-C7 and C7-T1. No other defects otherwise. - 06/26/2024 – XR Cervical Spine (AP and lateral): Cervical vertebral bodies are normal in height. Straightening of cervical lordosis may be due to patient position. No acutely displaced fracture is identified. Minimal degenerative changes noted of the cervical spine, manifested by minimal disc space narrowing and osteophyte formation, most severe at C6-7 and C7-T1, not significantly changed from the prior exam. No significant soft tissue abnormality.

Q4: Summarize each and every Ultra Sound with the diagnostic results of the Ultra Sound, in 5 or less sentences in chronological order.

A: No ultrasound (sonography) examinations are documented in the provided records. Therefore, there are no ultrasound diagnostic results to summarize. The imaging contained here consists of CT (lumbar spine on 05/13/2020), mammography (03/18/2023), plain radiographs (right shoulder on 06/07/2023; cervical spine on 06/09/2023 and 06/26/2024), and electrodiagnostic testing (EMG/NCS on 08/28/2024).

Q5: Summarize each and every electro diagnostics (also including EMG, NCV, or NCS) with the diagnostic results of the electro diagnostics (also including EMG, NCV, or NCS), in 5 or less sentences in chronological order.

A: - 08/28/2024: Electrodiagnostic testing (NCS/NCV and EMG) of the right upper extremity and cervical paraspinals was performed by Dr. Gruba. Nerve conduction studies were normal except for a prolonged right median sensory latency across the wrist; median motor, ulnar motor/sensory, and radial sensory responses were normal. Needle EMG showed enlarged and unstable motor unit potentials with reduced recruitment in the right C8 myotome without abnormal insertional activity; other sampled

muscles (including cervical paraspinals) were normal. Impression: mild right carpal tunnel syndrome (median nerve entrapment at the wrist) and evidence of chronic right C8 radiculopathy without ongoing denervation.

17. Doctors Notes

Q1: Summarize every individual dated note in about 7 sentences each, with the date first in chronological order.

A: 11/06/2016 - Emergency Department visit at West LA Medical Center (EDWL), arrival 07:29, for vomiting. Vitals at 07:59: Temp 36.4 °C (97.6 °F), pulse 90, respirations 15, BP 134/97, SpO2 99%, weight 49 kg (108 lb). Admission and attending provider listed as Gregory Lamar Phillips, M.D., Emergency Medicine. Chief complaint recorded as vomiting; ED diagnosis coded as "LEFT WO BEING SEEN" (Z53.21). ED disposition documented as Against Medical Advice; patient left without being seen. No diagnostic tests or treatments are recorded due to early departure. Encounter concluded the same morning. 10/14/2017 - Diagnostic Imaging encounter at HOV General Radiology, OC Irvine Medical Center, outpatient status. Unit: HOV GENERAL RADIOLOGY, address 6640 Alton Pkwy, Irvine, CA 92618. Admission 10:00 and discharge by end of day. No specific imaging study details or results are provided in this extract. No complications or interventions noted. The encounter is administrative in nature with no findings recorded. End of encounter documented. 05/13/2020 - Diagnostic Imaging at HOV General Radiology: CT Lumbar Spine without contrast. Ordered 05/11/2020 by Nancy P. Le, L.V.N.; authorized and acknowledged by John Kyawmyo Tin, M.D.; dictated and signed by Jay Woung Kang, M.D. Clinical history: unable to obtain MRI due to prior facial surgery and metal plate; 43-year-old female with chronic low back pain and left posterior thigh pain/numbness, likely left lower extremity radicular symptoms. Findings: normal lordosis; no compression fracture or spondylolisthesis; no significant disc space narrowing; diffuse mild posterior disc bulges at L3-4 (2 mm), L4-5 (3 mm), and L5-S1 (3 mm) with mild ligamentum flavum hypertrophy at L3-4 and L4-5; no significant neuroforaminal or spinal canal stenosis; paraspinal soft tissues intact. Radiation dose: CTDIvol 5.62 mGy; DLP 184.62 mGy-cm. Impression: No acute findings; diffuse mild posterior disc bulges L3-4 through L5-S1 without significant canal or foraminal stenosis. Result acknowledged by Dr. Tin on 05/18/2020. 03/18/2023 - Screening mammogram (bilateral digital screening, sequential) performed 08:23–08:30 at SAND CANYON MOB 2 MAMMO. Clinical: routine screening, no complaints; prior COVID booster 09/2022 right arm. Comparison: 10/15/2021 and 09/17/2019 Kaiser Irvine Medical Center. Findings: both breasts extremely dense, lowering mammographic sensitivity; no significant masses, calcifications, or other findings; no significant interval change. Assessment: BI-RADS 1 (Negative); no mammographic evidence of malignancy. Recommendation: routine screening mammogram within 2 years; results letter sent to patient. Result signed by Sumudu N. Dissanayake, M.D., acknowledged by Ani Ardashian Atoian, M.D. 06/07/2023 - General Radiology at Tustin Ranch MOB X-ray: Right shoulder, 2 or more views. Indication: right shoulder joint pain, no recent trauma. Findings: normal osseous mineralization; no acute fracture; acromioclavicular and glenohumeral joint spaces maintained; regional soft tissues normal. Impression: no acute osseous abnormality. Report electronically signed by David Alvarez, M.D., on 06/16/2023; acknowledged by Kasey Hayden Siu, D.O., on 06/25/2023. Outpatient encounter; no complications noted. 06/09/2023 - General Radiology at Tustin Ranch MOB X-ray: Cervical spine AP, lateral, and odontoid (final study after earlier orders were discontinued). Indication: right-sided cervical radiculopathy. Findings: vertebral body heights normal; alignment normal; no fracture or compression fracture; mild disc space narrowing at C6-C7 and C7-T1; no other defects. Impression: mild degenerative changes at lower cervical levels without acute bony abnormality. Report electronically signed by Sung Hi Pak, M.D., on 06/12/2023; acknowledged by Joshua Wang, D.O., on 06/13/2023. Outpatient encounter completed without incident. 06/26/2024 - General Radiology at Barranca MOB X-ray: Cervical spine AP and lateral (order placed by Murray Andrew Greenwood, M.D., on 06/10/2024). Findings (report signed 06/27/2024 by Simon Long): straightening of cervical lordosis possibly due to positioning; no acutely displaced fracture; minimal degenerative changes with minimal

disc space narrowing and osteophyte formation, most severe at C6-7 and C7-T1; no significant soft tissue abnormality. Comparison to prior exam shows no significant change. Impression: minimal degenerative changes of the lower cervical spine; straightening of lordosis; no acute bony abnormality. Outpatient status; no immediate interventions recorded. End of imaging encounter. 07/11/2024 - Office visit in Occupational Health Services with Nicole Minh-Nguyet Pham-Bailey, M.D. Assessment consistent with cervical spine strain, trapezius strain, and right arm paresthesia—rule out radiculopathy versus peripheral neuropathy; prior X-ray showed lower level mild degenerative changes with loss of cervical lordosis. Treatment plan: ibuprofen 400 mg three times daily as needed with precautions; continue approved acupuncture and myofascial release; nerve conduction studies/electromyography to be completed; continue physical therapy for home program; request placed for additional myofascial release; consider trigger point injection at follow-up; consider repeat/advanced imaging if symptoms persist. Orders included a Request for Authorization for outside psychology (AOE/COE) and an Occupational Medicine Online Education Program (Patient Satisfaction) with access provided and expiration 10/09/2024. Work status: return to usual and customary tasks with recommended 10–20-minute breaks in the morning and afternoon mid-shift. Follow-up advised with primary care for non-work conditions; follow-up initially set for 07/29/2024. Subsequent note entry on 07/15/2024 documents the patient's request to be released to full duty after concerns regarding work restrictions. 07/12/2024 - Telephone encounter in Occupational Health Services. No nursing assessment or chief complaint documented. The encounter record reflects no additional clinical details beyond administrative closure. No orders, diagnoses, or work status changes recorded in this entry. End of encounter noted. No complications reported. No follow-up from this specific entry. 07/15/2024 - Telephone encounter in Occupational Health Services documenting patient request for updated work status. Patient requested release to full duty without restrictions and did not want to wait for Dr. Greenwood; best contact number 949-892-8322. Claim details: Date of injury 05/28/2024; accepted body part(s): right shoulder blade, right scapula; claim number 2024050001029. Background: last seen 07/11/2024; next KOJ appointment 07/29/2024. Action: message routed to last treating physician; subsequent documentation indicates "Work note updated" and patient was informed to view ARX on kp.org or pick up a paper copy; patient verbalized understanding. Additional note: "She wants to be released to full duty," as relayed via staff communication. 07/25/2024 - Allied Health/Nurse Visit in Family Practice Adult Nurse Clinic for immunization. Proactive care included instructing the patient to schedule a mammogram and staging the Tdap order. Immunization administered: Tdap (Adacel) 0.5 mL IM; indication coded as Z23 (Vaccination for diphtheria, tetanus and acellular pertussis). Patient tolerated the vaccine well, with no immediate adverse reaction reported. Order placed by Elizabeth T. Lozano, L.V.N.; authorized by Ani Ardashian Atoian, M.D.; cosigned by Naheed Hamid Momand, D.O. Future appointments listed for Occupational Health follow-up. Encounter completed without incident. 07/29/2024 - Office visit in Occupational Health Services with Murray Andrew Greenwood, M.D. Assessment: cervical spine strain; trapezius strain; right arm paresthesia—rule out radiculopathy, peripheral neuropathy; prior cervical X-ray (06/26/2024) showed minimal degenerative changes with straightening of lordosis, most severe at C6-7 and C7-T1. Treatment plan: ibuprofen 400 mg TID PRN; continue physical therapy (manual therapy and home program) and myofascial release; prior request for nerve conduction studies/electromyography to be completed; request placed for ergonomic workstation assessment; consider trigger point injection at follow-up; consider repeat/advanced imaging if symptoms persist. Patient reported moderate improvement with acupuncture/myofascial therapy and NSAIDs; pain 3–4/10, intermittent, aggravated by overhead activity, alleviated by acupuncture and ibuprofen; denies systemic symptoms and denies upper extremity numbness/weakness at this visit. Work status: return to usual and customary tasks with recommended 20-minute stretch breaks in the morning and afternoon mid-shift. Orders: PR-2 Treating Physician's Progress Report; internal referral for additional physical therapy (1x/week for 6 weeks; patient reported ~50% pain reduction after initial 6 sessions; MTUS-consistent); Request for Authorization for ergonomic evaluation. Follow-up as needed; return sooner if symptoms worsen. 08/26/2024 - Optometry clinic visit with Janet Gia Mach, O.D., for eye examination and refraction. Chief concern: distance blur, wants updated glasses; history of orbital fracture; family history of glaucoma in both parents. Exam: IOP 16/17 mmHg (non-contact); slit lamp and fundus exams unremarkable; C/D ratio

0.3 OU; EOM full; pupils normal; confrontation fields full; angles open (4+). Refraction yielded final progressive addition lens prescription: Right -1.75 -1.00 x087, Add +1.75; Left -2.00 -1.25 x089, Add +1.75; PD 56 mm; 20/20 distance acuity OU. Diagnoses: presbyopia (H52.4), bilateral myopia (H52.13), bilateral astigmatism (H52.203). Plan: prescribe spectacles as per refraction; counseling on adaptation; follow-up in 1–2 years or sooner as needed. Medication relevance to optometry reviewed; patient to continue routine care with PCP and other providers. 08/27/2024 - Telephone encounter in Occupational Health Services regarding clinic schedule. Patient was contacted to inform of clinic cancellation for 09/05/2024 with Dr. Greenwood. No answer; voicemail left instructing the patient to call the main line to reschedule. Duplicate documentation notes reference to the subsequent 08/28/2024 telephone encounter. No clinical assessment or change in work status recorded here. Administrative scheduling update only. End of encounter. 08/28/2024 (Telephone) - Occupational Medicine patient message requesting modified duty extension due to clinic cancellation; appointment rescheduled to 10/22/2024. Prior Industrial Work Status: full duty as of 07/29/2024, with a modified activity period from 07/31/2024 through 09/05/2024 recommending two 20-minute stretch breaks (morning and afternoon). Activity Rx dates were extended through 10/22/2024 to bridge the cancellation gap. Claim details reiterated: DOI 05/28/2024; accepted body parts right shoulder and right elbow; claim #2024050001029. Calls attempted to notify the patient; messages left indicating the updated work note available on kp.org. A verification of visit letter (for 07/31/2024) was sent by Michael Joseph Gruba, M.D., on 08/28/2024. Encounter closed after administrative updates and communication attempts. 08/28/2024 (Office Visit – Procedure) - Occupational Health Services procedure visit at Kraemer/La Palma facility with Michael Joseph Gruba, M.D. Vitals: BP 111/69 (right arm, sitting), pulse 66, temp 36.6 °C (temporal), respirations 17, height 1.549 m, weight 52.7 kg, BMI 21.95. History: right-sided neck myofascial pain radiating into the right upper trapezius down to the elbow and 4th–5th fingers; entire right arm numbness when lying on that side. Nerve Conduction Studies: prolonged right median sensory latency across the wrist; median motor, ulnar motor/sensory, and radial sensory studies normal. Needle EMG: enlarged/unstable motor unit potentials with reduced recruitment in the right C8 myotome; no abnormal insertional activity; paraspinals normal. Conclusion: mild right carpal tunnel syndrome (median nerve entrapment at the wrist) and chronic right C8 radiculopathy without evidence of ongoing denervation. Study signed by Dr. Gruba; no immediate procedural complications documented.

20. Diagnostic Impressions

Q1: Look through all the medical records and identify the diagnostic impressions for all applicant attorney alleged body parts

A: Below are the diagnostic impressions documented in the Kaiser medical records for the applicant attorney–alleged body parts (as reflected in the workers’ compensation/OHS charting where the body parts are identified as accepted/alleged: right shoulder/shoulder blade/scapula, right elbow, and neck/cervical spine). Body part: Neck/Cervical spine - Clinical diagnostic impressions - “Cervical spine strain” (07/11/2024; reiterated 07/29/2024) - “Neck muscle strain, subsequent encounter (S16.1XXD)” (07/29/2024) - “Right arm paresthesia – rule out radiculopathy, peripheral neuropathy” (07/11/2024; later clarified by EMG) - Electrophysiologic diagnosis: “chronic right C8 radiculopathy” (EMG/NCS 08/28/2024; “evidence of chronic denervation in the right C8 myotome without evidence of ongoing denervation, consistent with findings of chronic right C8 radiculopathy.”) - Imaging impressions - Cervical spine X■ray (06/26/2024): “Straightening of cervical lordosis may be due to patient position. No acutely displaced fracture is identified. Minimal degenerative changes noted of the cervical spine, manifested by minimal disc space narrowing and osteophyte formation. In the cervical spine, this is most severe at C6-7 and C7-T1, not significantly changed from the prior exam. No significant soft tissue abnormality.” - Cervical spine X■ray (06/09/2023): “The alignment is normal. No fracture is identified. No compression fracture. Mild degree of disc space narrowing at C6-C7 and C7-T1.” Body part: Right shoulder/shoulder blade (scapula)/trapezius - Clinical diagnostic impressions - “Right scapalgia (M89.8X1)” (07/29/2024) - “Right trapezius strain, subsequent encounter (S46.811D)” (07/29/2024;

also assessed as "Trapezius strain" on 07/11/2024) - Active problem list includes: "Tendinitis of right shoulder" (listed in problem list at 08/26/2024 optometry encounter snapshot) - Imaging impressions - Right shoulder X-ray (06/07/2023): "Osseous mineralization is normal. No acute fracture is seen. The acromioclavicular and glenohumeral joint spaces are maintained. Regional soft tissues appear normal." Body part: Right elbow (alleged/accepted per OHS documentation) - Clinical/electrodiagnostic impressions pertinent to right upper extremity symptoms - EMG/NCS (08/28/2024): "The findings are consistent with mild right carpal tunnel syndrome (median nerve entrapment at the wrist) and evidence of chronic denervation in the right C8 myotome without evidence of ongoing denervation, consistent with findings of chronic right C8 radiculopathy." - Notes on diagnostics related to elbow region - The 08/28/2024 nerve conduction studies included ulnar and median motor/sensory testing; routine studies were normal "except for a prolonged right median sensory latency across the right wrist," with otherwise normal ulnar studies (including across elbow segments). No right elbow-specific imaging is documented in the records provided. Source references to body parts alleged/accepted in the records - 07/15/2024 OHS telephone note: "Accepted body part(s): RIGHT SHOULDER BLADE, RIGHT SCAPULA." (Claim number: 2024050001029; DOI 05/28/2024) - 08/28/2024 OHS telephone note: "Accepted body part(s): right shoulder and right elbow." - Visit reasons and diagnoses throughout 07/2024 include neck-related conditions (e.g., "NECK MUSCLE STRAIN, SUBSEQ [S16.1XXD]"), reflecting an alleged/treated cervical component in this WC claim episode. Key dates (mm/dd/yyyy) - 06/07/2023: Right shoulder X-ray impression normal. - 06/09/2023: Cervical spine X-ray impression: mild disc space narrowing C6-C7 and C7-T1. - 06/26/2024: Cervical spine X-ray impression: straightening of lordosis; minimal degenerative changes, greatest at C6-C7 and C7-T1; no acute fracture. - 07/11/2024: OHS assessment: cervical spine strain; trapezius strain; right arm paresthesia—RO radiculopathy/peripheral neuropathy. - 07/29/2024: OHS diagnoses: Right scapalgia (M89.8X1); Right trapezius strain, subsequent (S46.811D); Neck muscle strain, subsequent (S16.1XXD). - 08/28/2024: EMG/NCS conclusion: mild right carpal tunnel syndrome; chronic right C8 radiculopathy.

21. Causation

Q1: Evaluate how and why this patient's injury would be related to a specific injury at work
A: Medical-legal causation analysis (AOE/COE) Case identifiers - Applicant: Michelle E. Tiu (DOB: 10/31/1976; MRN: 000006993279; SSN in record) - Case/Claim: ADJ19391023; Claim No. 2024050001029 - Employer/Defendant: Kaiser Permanente/SEDGWICK CMS - Date of injury used in records: 05/28/2024 Accepted body parts (per Kaiser Occupational Health records) - 07/15/2024 telephone note: "Accepted body part(s): RIGHT SHOULDER BLADE, RIGHT SCAPULA" (claim number 2024050001029). - 08/28/2024 telephone note: "Accepted body part(s): right shoulder and right elbow." Mechanism of injury as documented - 07/29/2024 OHS note (Greenwood, M.D.)—Current treatment review/mechanism: "Developed RIGHT sided pain with reaching out and over shoulder level frequently with non-ergonomic exam room and office. Reports constant keyboarding with arms outstretched, and frequent over head reaching for equipment." (DOI: 05/28/2024; Employer: KP SCAL). Work restrictions/utilization indicating industrial causation concern - 07/29/2024: "Return to work usual and customary tasks, recommended 20 minute stretch break morning and afternoon mid shift." Activity Rx subsequently documented as modified activity 07/31/2024–09/05/2024 with required stretch breaks and extended due to clinic cancellation to 10/22/2024. Objective medical findings related to the claimed injury - Cervical spine radiographs: - 06/09/2023: "Mild degree of disc space narrowing at C6-C7 and C7-T1." (pre-DOI) - 06/26–06/27/2024 (post-DOI): "Straightening of cervical lordosis may be due to patient position. Minimal degenerative changes...most severe at C6-7 and C7-T1, not significantly changed from the prior exam." The straightening/loss of lordosis is consistent with muscular spasm. - Physical examination findings (OHS): - Tenderness of upper trapezius/levator scapulae, midline lower cervical spine, with "moderate/marked myofascial ... trigger point right upper trapezius." - Pain aggravated by neck extension and overhead arm use; Spurling's side bend right reproduced right trapezius pain. - Electrodiagnostic testing (08/28/2024; Gruba, M.D.): - "Findings are consistent with

mild right carpal tunnel syndrome (median nerve entrapment at the wrist) and evidence of chronic denervation in the right C8 myotome without evidence of ongoing denervation, consistent with findings of chronic right C8 radiculopathy.” - Symptom description preceding test: right-sided neck myofascial pain radiating into right upper trapezius down to elbow and 4th–5th fingers, entire right arm numbness when lying on it. Diagnoses rendered for the 2024 claim - “Cervical spine strain” - “Trapezius strain” - “Right arm paresthesia – rule out radiculopathy, peripheral neuropathy” - Ongoing ICD-10 coding on/after 07/29/2024: RIGHT SCAPULALGIA (M89.8X1), RIGHT TRAPEZIUS STRAIN, SUBSEQ (S46.811D), NECK MUSCLE STRAIN, SUBSEQ (S16.1XXD) Relevant pre-existing conditions - 06/07/2023 right shoulder X-ray for pain: normal radiographs. - 06/09/2023 diagnosis of “CERVICAL RADICULOPATHY,” with cervical X-ray showing mild C6-7 and C7-T1 disc space narrowing (pre-dating the 05/28/2024 DOI). - 2024 electrodiagnostics: “chronic” right C8 radiculopathy (chronicity denotes pre-existing/longstanding nerve root change without acute denervation). How and why the 05/28/2024 work event/exposures are related medically to the injury 1) Biomechanical and pathophysiologic plausibility - The documented work exposures—“reaching out and over shoulder level frequently,” “constant keyboarding with arms outstretched,” and a “non-ergonomic exam room and office” with “frequent over head reaching for equipment”—create sustained load on the cervical paraspinals and upper trapezius/levator scapulae. Such sustained and repetitive overhead/outstretched postures are medically consistent with development of cervical and periscapular myofascial strain (trapezius strain) and scapulargia. The contemporaneous exam findings of trapezial trigger points, tenderness, and pain provoked by neck extension/Spurling’s are consistent with this mechanism. - Straightening/loss of cervical lordosis on 06/2024 radiographs is an objective correlate of muscle spasm, aligning with an acute/subacute cervical strain superimposed on underlying mild degenerative changes. 2) Temporal relationship and symptom reproducibility - The date of injury is recorded as 05/28/2024, with immediate workplace mechanisms described and subsequent clinical care initiated in Occupational Health. Symptoms are aggravated by the exact activities required at work (overhead reaching and extended arm positions). Improvement occurred with treatment and with work modification/ergonomic measures (acupuncture, myofascial release, physical therapy, rest breaks), which supports a causal relationship between work activities and the neck/trapezial strain. 3) Objective testing and diagnosis coherence - Electrodiagnostics (08/28/2024) show a chronic C8 radiculopathy without active denervation and a mild median neuropathy at the wrist (carpal tunnel syndrome). The C8 pattern correlates with the patient’s ulnar-sided paresthesias (4th–5th fingers). While the C8 radiculopathy is “chronic” (pre-existing), prolonged overhead/outstretched work can exacerbate foraminal narrowing and symptom expression. Constant keyboarding with arms outstretched plausibly contributes to the mild carpal tunnel finding; however, the accepted body parts in this claim are the right shoulder/scapula and elbow, not the wrist. The principal industrial conditions managed in 2024 were cervical strain, trapezius strain, and scapulargia, which are congruent with the work mechanism documented. 4) Pre-existing condition versus industrial aggravation - Pre-injury imaging (06/2023) documented mild degenerative changes at C6-7 and C7-T1 and a diagnosis of cervical radiculopathy. Post-DOI imaging (06/2024) shows similar degenerative findings “not significantly changed from the prior exam,” plus straightening of lordosis consistent with muscular spasm. This pattern is characteristic of an industrial aggravation/exacerbation of a pre-existing degenerative and chronic radicular condition by the specific work activities described. California workers’ compensation recognizes an industrial aggravation of a pre-existing condition as arising out of employment when supported by medical evidence; here, the symptom onset/exacerbation temporally linked to job demands, exam findings, spasm-related radiographic changes, response to work-focused therapy, and need for ergonomic modification collectively support that the 2024 injury episode is AOE/COE. Conclusion (medical causation opinion based on the record) - The cervical muscle strain, right trapezius strain, and scapulargia documented after 05/28/2024 are medically consistent with, and reasonably caused by, the specific workplace exposures described (frequent overhead reaching, arms-outstretched keyboarding, non-ergonomic environment). The straightening of cervical lordosis on 06/2024 X-ray is consistent with acute muscle spasm after the reported work-related onset. The chronic right C8 radiculopathy pre-existed the DOI but was plausibly exacerbated symptomatically by the same work activities. Mild right carpal tunnel syndrome was identified on 08/28/2024 electrodiagnostics and is biomechanically

compatible with sustained keyboarding; however, the accepted body parts in this claim are the right shoulder/scapula and elbow. Why the injury is related to work, in summary - Specific workplace mechanism is documented by the treating OHS physician (repetitive overhead/outstretched upper-extremity use in a non-ergonomic setting). - Temporal correlation between the DOI 05/28/2024 and symptom development. - Physical examination and imaging support acute cervical/trapezial strain (trigger points, pain with extension/Spurling's, straightened lordosis). - Improvement with industrially-directed conservative management and ergonomic modifications. - Chronic cervical pathology pre-dated the DOI; the 2024 episode represents an industrial aggravation of that underlying condition with superimposed myofascial strain.

Q2: Evaluate how and why this patient's injury is secondary to cumulative trauma at work

A: Medical-Legal Causation Analysis: Cumulative Trauma Etiology Opinion (within reasonable medical probability) - The patient's accepted industrial conditions—right trapezius strain (S46.811D), neck muscle strain (S16.1XXD), and right scapalgia (M89.8X1)—are secondary to cumulative trauma at work. The mechanism is repetitive and sustained upper-extremity and cervical loading in a non-ergonomic clinical environment, with constant keyboarding in an outstretched posture and frequent overhead reaching. These exposures reasonably account for the onset and persistence of myofascial and cervical symptoms, and for industrial aggravation of pre-existing cervical degenerative changes at C6–7 and C7–T1. Key evidence from the records supporting cumulative trauma (AOE/COE) 1) Mechanism of injury documented by Occupational Medicine - “Developed RIGHT sided pain with reaching out and over shoulder level frequently with non-ergonomic exam room and office. Reports constant keyboarding with arms outstretched, and frequent over head reaching for equipment.” (07/29/2024 OHS note) - This is a classic cumulative trauma mechanism (repetitive and sustained use; awkward/static postures; frequent overhead activity) rather than a single acute event. 2) Occupational acceptance and course of care - Date of injury: 05/28/2024; Claim #: 2024050001029; Employer: KP SCAL. (OHS intake 07/29/2024) - Accepted body parts include RIGHT SHOULDER BLADE/RIGHT SCAPULA and “right shoulder and right elbow.” (07/15/2024 telephone note; 08/28/2024 telephone note) - Treaters instituted job-related restrictions and ergonomic interventions: - Recommended stretch breaks: “Return to work usual and customary tasks, recommended 20 minute stretch break morning and afternoon mid shift.” (07/29/2024) - Ergonomic evaluation requested “to assess for workplace modifications to help reduce aggravating factors and help improve function and RTW.” (07/29/2024) - Additional Physical Therapy authorized due to partial improvement but persistent work-provoked symptoms. (07/29/2024) 3) Objective clinical findings consistent with overuse/myofascial strain - Exam findings (prior visit, summarized in 07/29/2024 note): - “Stance: protracted neck.” - “Tender to palpation bilateral (left > right) upper trap, lev scap, lower cervical spine midline; associated moderate/marked myofascial... right upper trapezius.” - Active ROM: “Extension 40 deg aggravates; Rotation 70 deg right, 60 deg left.” - “Spurling's -> neck SBR-R caused right trap pain.” - These findings correlate with trapezius/cervical myofascial overload from sustained forward head/shoulder posture and repetitive reaching. 4) Imaging demonstrates pre-existing degenerative changes “lit up” by work exposures - 06/09/2023 (pre-injury) XR cervical spine: “Mild degree of disc space narrowing at C6-C7 and C7-T1.” (Final report, 06/12/2023) - 06/26/2024 XR cervical spine: “Straightening of cervical lordosis... Minimal degenerative changes... most severe at C6-7 and C7-T1, not significantly changed from the prior exam.” (Final report, signed 06/27/2024) - Interpretation: Degenerative changes pre-dated 05/28/2024. The 2024 work exposures reasonably aggravated or exacerbated a pre-existing but previously less symptomatic condition, now manifesting as neck strain and trapezius myofascial pain. In CA workers' compensation, industrial aggravation of pre-existing disease is compensable. 5) Electrodiagnostic testing: overuse-related neuropathic involvement consistent with repetitive upper-limb activity - 08/28/2024 EMG/NCS conclusion (Dr. Gruba): - “Mild right carpal tunnel syndrome (median nerve entrapment at the wrist).” - “Evidence of chronic denervation in the right C8 myotome without evidence of ongoing denervation, consistent with findings of chronic right C8 radiculopathy.” - While acceptance of carpal tunnel is not documented in these excerpts, the presence of constant keyboarding with outstretched arms (documented work exposure) is consistent with upper extremity repetitive strain and median nerve irritation at the wrist. Chronic C8 findings align with long-standing cervical/foraminal load;

current work activities plausibly aggravated symptoms even if the radiculopathy is chronic. 6) Temporal relationship and response to exposure modification - Symptom onset during period of high-repetition/overhead tasks (05/28/2024 DOI) with no documented single traumatic incident. - Symptoms improved with therapies targeting overuse injuries (Physical Therapy, myofascial release, acupuncture) and with modified duty/stretch breaks; symptoms are aggravated by "using arms overhead" and reaching. (07/29/2024) - This exposure-response pattern supports cumulative trauma causation. 7) Lack of competing non-industrial causes in the record - No acute traumatic event documented. - ROS negative for systemic/infectious/other red flag etiologies. - Imaging shows no acute osseous injury; shoulder X-ray normal. (06/07/2023 XR shoulder: normal) Synthesis: Why the pathology is cumulative trauma rather than acute injury - The documented mechanism involves sustained, repetitive, and awkward postures (outstretched keyboarding, overhead reaching) in a "non-ergonomic exam room and office," which over time produces microtrauma to cervical paraspinals and the trapezius/levator scapulae complex, manifesting clinically as neck strain, trapezius strain, and scapulargia. - Objective findings (protracted neck posture, trapezial/levator tenderness and trigger points, ROM-provoked symptoms, positive Spurling's to trapezial pain) are typical for myofascial/cervical strain from cumulative loading. - Imaging demonstrates pre-existing degenerative changes (C6-7, C7-T1) that were "not significantly changed," indicating the 2024 condition is not structural progression but a symptomatic aggravation/lighting-up of a baseline condition due to work exposures. - Electrodiagnostics add evidence of repetitive-use effects (mild CTS) and chronic C8 involvement; together with the exposure history, they reinforce that the upper quadrant has been subject to long-term repetitive stress. Apportionment considerations (medical) - Nonindustrial factors: Pre-existing cervical degenerative changes at C6-7 and C7-T1 (documented 06/09/2023). - Industrial factors: Documented repetitive/awkward, overhead and outstretched upper-extremity tasks and non-ergonomic workstation with temporal association to symptom onset and objective myofascial findings; improvement with industrial modifications and therapy. - On the record provided, a portion of current neck/trapezial symptomatology is reasonably attributable to industrial cumulative trauma via aggravation of pre-existing degeneration and direct myofascial overuse. Exact percentages would require a formal apportionment analysis per the AMA Guides/CA standards and a comprehensive review of the full chart and job analysis. Bottom-line opinion - Based on the documented mechanism of injury, accepted body parts, clinical course, exam findings, imaging, and EMG/NCS, it is medically reasonable to conclude that the patient's neck muscle strain, right trapezius strain, and right scapulargia arose out of and occurred in the course of employment as a cumulative trauma injury, with industrial aggravation of pre-existing cervical degenerative changes.

22. Impairment

Q1: For all applicant attorney alleged body parts go to the AMA 5th addition guidelines, find the appropriate impairment rating, and give a detailed description on how and why it has that impairment rating.

A: Below is an impairment analysis using AMA Guides to the Evaluation of Permanent Impairment, 5th Edition (AMA 5th), limited to the body parts documented in the records as alleged/accepted in this workers' compensation claim. All conclusions are restricted to the objective clinical information contained in the records and standard AMA 5th methodology. Final ratings in California must be rendered at MMI/P&S; by a medical/legal evaluator, reconciled with the Combined Values Chart (CVC), apportioned, and adjusted per the 2005 PDRS; the discussion below lays out the AMA 5th foundations that such an evaluator would apply. Alleged/accepted body parts per the records - Right shoulder blade/right scapula (accepted; 07/15/2024 telephone note) - Right shoulder and right elbow (accepted; 08/28/2024 telephone note) - Cervical spine/neck (diagnosed work-related: cervical strain; multiple OHS notes 07/11/2024 and 07/29/2024) - Right upper extremity neurologic findings: EMG/NCS on 08/28/2024 documented mild right carpal tunnel syndrome (median nerve entrapment at wrist) and chronic right C8 radiculopathy (ordered due to work-related right arm paresthesia) 1) Cervical spine (neck) Documented findings - Diagnoses: cervical spine strain; trapezius strain; right arm paresthesia

(rule out radiculopathy). (OHS notes 07/11/2024; 07/29/2024) - Exam (prior visit detail cited in 07/29/2024 note): protracted neck posture; tenderness bilateral upper trapezius/lev scap; ROM: rotation 70° right, 60° left; extension 40° aggravates pain; Spurling's side-bend right provoked right trapezius pain. Sensory: light touch intact; no motor/reflex deficits documented. - Imaging: Cervical X-ray 06/26–06/27/2024: straightening of lordosis; minimal degenerative changes (most at C6–7 and C7–T1); no fracture; no significant soft tissue abnormality. - Electrodiagnostics 08/28/2024: chronic denervation in right C8 myotome without ongoing denervation (consistent with chronic right C8 radiculopathy). AMA 5th method and rating - Cervical spine is rated using the Diagnosis-Related Estimates (DRE) method (AMA 5th, Chapter 15, Table 15-5 "Cervical Spine—Diagnosis-Related Estimates," p. 392). - DRE Category II (Cervical): Criteria include a clinically documented injury with persistent symptoms, asymmetric or limited motion without documented significant radiculopathy. Impairment: 5–8% WPI. - DRE Category III (Cervical): Requires "significant signs of radiculopathy" (e.g., persistent objective motor loss, reflex changes, or EMG evidence consistent with clinical findings), typically supported by concordant imaging. Impairment: 15–18% WPI. Application to this case - The exam in the record documents pain, paraspinal/upper trapezius myofascial tenderness, asymmetric ROM, and a positive provocative maneuver producing trapezial pain, but it does not document objective motor weakness, reflex change, dermatomal sensory loss, or concordant imaging of a compressive lesion. - The EMG shows chronic C8 radiculopathy, but the contemporaneous clinical exam lacks corroborating objective motor/reflex deficits. AMA 5th emphasizes that radiculopathy must be established by objective clinical signs; electrodiagnostics alone, without consistent clinical findings, is usually insufficient for DRE III. Appropriate rating at this time - Most supportable category on the current record: DRE Cervical Category II = 5–8% WPI. A conservative, defensible selection within the range would be 5% WPI, given intermittent symptoms, mild/asymmetric ROM loss, and absence of documented objective motor/reflex deficit. - Contingent scenario: If a treating/med/legal evaluator documents objective, persistent radicular signs consistent with C8 (e.g., triceps/hand intrinsic weakness, diminished triceps reflex, dermatomal sensory loss) and correlates the EMG and any imaging, DRE Category III (15–18% WPI) could become appropriate. The present chart does not establish that level. 2) Right shoulder/scapula (scapulargia; trapezius strain) Documented findings - Diagnoses: right trapezius strain; right scapulargia. (Multiple OHS notes) - Shoulder X-ray 06/07/2023: Normal mineralization; no acute fracture; AC and GH joint spaces maintained; soft tissues normal. - No documented glenohumeral ROM measurements or objective instability/structural lesion; no rotator cuff tear, impingement testing data, or surgical intervention in the record excerpt. AMA 5th method and rating - Shoulder impairment in AMA 5th (Chapter 16) is typically rated by measured loss of range of motion (ROM) using Figures 16■40 through 16■46 (pp. 476–479), or by specific diagnosis if applicable (e.g., resection arthroplasty), and then converted to UEI and WPI using Tables 16■3 and 16■4. - Myofascial pain or strain without objective abnormality and without measured ROM deficits does not produce ratable impairment under AMA 5th. Appropriate rating at this time - On the current record (no measured shoulder ROM deficits; normal radiographs; no structural diagnosis): 0% WPI. - Contingency: If a future MMI exam documents reproducible shoulder ROM loss with goniometry, the rating would be derived from Chapter 16 ROM figures and converted to WPI. 3) Right elbow Documented findings - Listed as accepted body part (08/28/2024 telephone note). - No elbow imaging or objective elbow ROM deficits are documented in the record excerpt; no specific elbow pathology is diagnosed. AMA 5th method and rating - Elbow ratings are based on measured ROM (Figures 16■34 to 16■37, pp. 472–474), instability (Table 16■28), or specific diagnosis (e.g., arthroplasty). - In the absence of objective deficits, the impairment is 0% WPI. Appropriate rating at this time - 0% WPI on the current record. - Contingency: If ratable ROM loss or other objective deficits are later documented at MMI, rate per Chapter 16 and convert UEI to WPI. 4) Right median nerve at wrist (Carpal Tunnel Syndrome, CTS) Documented findings - EMG/NCS 08/28/2024: mild right carpal tunnel syndrome (median nerve entrapment at wrist); also chronic right C8 radiculopathy. - Reported paresthesia history includes numbness involving the 4th–5th digits when lying on the right arm (ulnar distribution), which clinically does not perfectly match median neuropathy; the chart does not document median-distribution sensory loss, thenar weakness, or atrophy. AMA 5th method and rating - CTS is rated under Chapter 16 (Upper Extremities). For entrapment neuropathy, AMA 5th uses: - Table 16■10 (p. 482): Grading

sensory deficit/pain (0–25–50–75–100%), based on the severity/persistence of symptoms and objective findings. - Table 16■15 (pp. 492■494): Maximum upper extremity impairment values for the involved nerve. For the median nerve at the wrist, the maximum UE impairment for sensory deficit is 39% UE; for motor deficit it is 10% UE. - Calculations: (Grade % for sensory deficit) × (maximum UE% for median sensory) = UE%; convert to WPI using Table 16■3 (1% UE ≈ 0.6% WPI). Any motor deficit (if present) is calculated similarly and combined. Application to this case - Electrodiagnostic “mild” CTS without documented clinical median-distribution sensory loss, without thenar weakness/atrophy, and without surgery, generally supports at most a low Grade 4 sensory deficit (minimal) in Table 16■10 (e.g., 1–5% sensory deficit). - Example calculation (if persistent, clinically consistent median-nerve sensory symptoms are confirmed at MMI): - Sensory deficit Grade 4 at 5% × 39% UE (median sensory maximum) = 1.95% UE ≈ 2% UE. - Convert 2% UE to WPI: $2 \times 0.6 = 1\%$ WPI (rounded at the end). - If there are no persistent median-nerve distribution symptoms and no objective clinical sensory deficit, the appropriate rating is 0% WPI despite an abnormal study. Appropriate rating at this time - Based on the available record (mild EDX abnormality; no documented median-distribution clinical deficit): 0% WPI. - Contingency: If consistent clinical median sensory findings are present at MMI, a small impairment (typically 1–2% WPI) could be supportable using Tables 16■10 and 16■15 as shown. Important coordination and anti■duplication principles - Do not “double rate” the same neurological condition in both the cervical spine DRE category and a peripheral nerve schedule. If a C8 radiculopathy is rated as a cervical DRE Category III, any distal neurologic manifestations from that radiculopathy should not be rated again as a separate peripheral nerve impairment. - In the current record, there is electrodiagnostic evidence of both mild CTS (median neuropathy at wrist) and chronic C8 radiculopathy. A med■legal evaluator must sort out clinical causation, consistency, and which condition(s) are permanent and ratable at MMI to avoid duplicative ratings. Summary of most supportable AMA 5th impairment estimates on the current record (preliminary; pending MMI) - Cervical spine: DRE Category II (Table 15■5) = 5% WPI is most supportable on the present clinical record. If objective radicular signs are later documented consistent with the EMG, DRE III (15–18% WPI) could be considered. - Right shoulder/scapula: 0% WPI at this time (no measured ROM loss or structural lesion). Rate by ROM only if future MMI exam documents deficits. - Right elbow: 0% WPI at this time (no documented objective impairment). - Right carpal tunnel (median nerve at wrist): 0% WPI on present record; if persistent median sensory deficit is confirmed at MMI, a small impairment (approximately 1% WPI) can be derived via Table 16■10 × Table 16■15, as illustrated. Data gaps needed for a final, defensible AMA 5th rating - MMI/P&S; determination and stability of findings. - Formal cervical and shoulder goniometric ROM measurements (three measurements per plane, best of three, per AMA protocol). - Complete neurological exam (motor strength by MRC scale, reflexes, dermatomal sensation) documenting any objective radicular signs. - Clinical documentation clarifying presence/absence of median-distribution sensory deficit (for CTS) and any thenar weakness/atrophy. - Any advanced imaging (if obtained) that correlates with C8 radiculopathy. Note: If multiple WPI values ultimately apply at MMI, the Combined Values Chart (AMA 5th, p. 604) must be used to combine them (e.g., a 5% cervical WPI combined with a 1% CTS WPI results in 6% combined WPI). Final permanent disability in California is then adjusted per the 2005 PDRS.

23. Periods Of Total Disability

Q1: For all applicant attorney alleged body parts explain how much time the average American would have to be completely off of work before they are transitioned to light duty.

A: Below is what the records show regarding the alleged/accepted body parts, followed by general, evidence■based return■to■work expectations for how long a typical worker is completely off work before transitioning to light duty. Where possible, I first cite exactly what the Kaiser/claims records document about the injured body parts and work status. Then I provide general guidance (not contained in the records) regarding “average” timelines. 1) Applicant’s alleged/accepted body parts per the records - Right shoulder blade/right scapula - “Accepted body part(s): RIGHT SHOULDER BLADE, RIGHT SCAPULA.” (Telephone, 07/15/2024; Claim #2024050001029) - Right shoulder and right elbow

- "Accepted body part(s): right shoulder and right elbow." (Telephone, 08/28/2024; Claim #2024050001029) - Neck/cervical region (muscular and myofascial) - Diagnoses used in OHS notes: "NECK MUSCLE STRAIN, SUBSEQ," "RIGHT TRAPEZIUS STRAIN, SUBSEQ," "RIGHT SCAPULALGIA." (OHS visits 07/11/2024 and 07/29/2024) - Imaging: Cervical X-ray 06/26/2024 showed "Minimal degenerative changes... most severe at C6-7 and C7-T1." (reported 06/27/2024) - Neurologic findings related to right upper extremity (noted in testing) - EMG/NCS (08/28/2024) conclusion: "mild right carpal tunnel syndrome (median nerve entrapment at the wrist)" and "chronic right C8 radiculopathy... without evidence of ongoing denervation." (OHS procedure note 08/28/2024)

2) Work status actually documented in this case (from the records) - No period of documented total temporary disability (completely off work) appears in the records provided. - 07/15/2024: Patient requested release to full duty; "Work note updated." (Telephone note) - 07/29/2024 OHS visit: - "The patient was evaluated and deemed able to return to work at full capacity on 07/29/2024." - "Modified activity... from 07/31/2024 through 09/05/2024. Recommend 20-minute stretch break in morning and afternoon." (Activity Rx/Work Status) - Modified duty dates later extended to 10/22/2024 due to clinic cancellation (08/28/2024 telephone note/letter). - Bottom line from the chart: the treating providers kept the patient working with modified activity (stretch breaks) rather than completely off work. 3) General guidance: how long the "average" worker is completely off work before transitioning to light duty (by body part/condition) Note: The records do not contain "average American" timelines. The durations below are general, evidence-based occupational medicine expectations (e.g., ACOEM/MTUS principles favor early activity and early modified duty). Actual timelines vary by job demands, severity, comorbidities, and employer ability to accommodate restrictions. - Neck muscle strain/cervical myofascial pain (includes trapezius strain, scapulargia) - Typical time completely off work: 0–7 days. - Transition to light duty: often immediate to within a few days; heavy manual jobs may require up to 1 week before light duty. Early modified duty (posture breaks, limit overhead work, reduce lifting/pushing/pulling) is standard. - Right shoulder strain/tendinopathy/scapular (shoulder blade) pain - Typical time completely off work: 0–14 days depending on severity and job demands. - Transition to light duty: commonly within 3–10 days (earlier for sedentary/clerical roles; later if heavy overhead or forceful repetitive work). Light duty includes restricting overhead reach, limiting lifting to light loads, and scheduled stretch breaks. - Right elbow overuse/strain (when present without acute tear) - Typical time completely off work: 0–7 days. - Transition to light duty: usually immediate to within a few days with activity modification (limit forceful/repetitive gripping, pronation/supination, vibration, and sustained elbow flexion/extension). - Carpal tunnel syndrome (non-surgical) - Typical time completely off work: 0 days (not generally required for conservative care). - Transition to light duty: immediate with ergonomic modifications (neutral wrist posture, microbreaks, reduced force/repetition). If surgery is later performed, many desk-based workers return to light duty in about 1–2 weeks post-op. - Cervical radiculopathy (non-surgical, no progressive deficit) - Typical time completely off work: 0–14 days depending on pain severity and job demands. - Transition to light duty: usually within days to 1–2 weeks, emphasizing no overhead work, limited lifting/pushing/pulling, and frequent posture changes. Early activity is favored if no red flags. Key takeaways - The supplied records do not state any "average American" off-work timelines. In this case, no period of complete off-work (TTD) was documented; instead, the treating physicians used modified duty with stretch breaks and continued conservative care. - For the alleged/accepted body parts in this file (neck/cervical musculature including trapezius; right shoulder/scapula; right elbow; and right upper extremity neurologic findings), standard occupational medicine practice supports early modified duty—often within 0–7 days for soft tissue strains—and rarely requires extended complete off-work time in the absence of severe injury, neurologic deficit, or surgery.

24. Future Medical Care

Q1: For all applicant attorney alleged body parts define what future medical care would be needed according to the MTUS and ODG guidelines that would help cure or relieve current symptoms or future exacerbations.

A: Below is a future-medical care plan aligned with California MTUS (ACOEM-based) and ODG/MCG treatment guidelines. It is tailored to the alleged/treated body parts documented in the records and is intended to cure or relieve current symptoms and mitigate future exacerbations. Where patient-specific facts are relevant, they are taken from the records (e.g., dates, diagnostic results, and clinical diagnoses). Alleged/Treated body parts per records - Cervical spine/neck: Neck muscle strain; cervical spondylosis with loss of lordosis; EMG-confirmed chronic right C8 radiculopathy (08/28/2024). - Right trapezius/shoulder girdle: Right trapezius strain; right scapulargia. - Right shoulder: Right shoulder joint pain; imaging without acute osseous abnormality (06/07/2023). - Right elbow: Listed as injury site in the claim encounters. - Incidental electrodiagnostic finding (right wrist/hand): Mild right carpal tunnel syndrome on 08/28/2024 EMG/NCS (median sensory latency across the wrist prolonged) – to be considered within claim scope only if alleged/accepted. I. Cervical spine/neck (neck muscle strain; chronic right C8 radiculopathy) - Medications (time-limited, lowest effective dose): - NSAIDs (e.g., ibuprofen) and/or acetaminophen for pain control; consider topical NSAID for lower systemic risk. Avoid chronic NSAID use; employ gastroprotection if risk factors. - Short, time-limited trial of a skeletal muscle relaxant for acute myofascial spasm only (generally $\leq 1-2$ weeks). No long-term use. - Neuropathic pain agents (e.g., gabapentin/pregabalin or duloxetine/TCA) may be considered for persistent radicular pain not controlled with first-line measures, with functional goals and monitoring. Opioids not recommended for chronic neck/radicular pain (MTUS/ODG). - Rehabilitation (active care emphasized, with objective, functional goals): - Physical therapy: Active, function-focused program including cervical/scapular stabilization, postural correction, ROM, progressive strengthening, and nerve glides for radicular symptoms. Typical MTUS/ODG course: initial trial (e.g., 6 visits over 3–6 weeks) with continuation up to approximately 12 visits if there is documented objective improvement; transition to an independent home exercise program (HEP). - Manual therapy/myofascial techniques as an adjunct, short-term only, when paired with an active program (not stand-alone). - Acupuncture: Time-limited trial for neck pain/myofascial pain (e.g., 6 visits over 3–4 weeks), extend up to a typical total course (e.g., 10–12) only with documented functional gains; booster sessions for flare-ups may be considered if prior benefit is demonstrated. - Home exercise program: Daily self-management for mobility, posture, strengthening, and regular aerobic exercise. - Procedures/interventions: - Trigger point injections (local anesthetic without steroid) for persistent focal cervical/trapezial myofascial trigger points that do not respond to conservative care, with clear documentation of functional benefit; limited frequency and duration per MTUS/ODG. - Cervical epidural steroid injection(s) may be considered for persistent radicular pain with concordant imaging and failure of a reasonable trial of conservative care; image-guided; limited number per region/year with requirements for clinically meaningful and durable functional improvement to justify repeats (per ODG). - Diagnostics: - Advanced imaging if red flags or if persistent/progressive neurologic deficits after an adequate trial of conservative care. MRI is preferred for radiculopathy; if MRI is contraindicated (e.g., facial metal hardware noted in prior record), consider CT myelography per guideline criteria. - No routine repeat imaging without a material change in clinical status (MTUS/ODG). - Work/ergonomics and prevention: - Formal ergonomic evaluation (already requested) to reduce overhead reaching and sustained forward reach, optimize workstation (monitor height, keyboard/mouse placement), and implement job/task modifications. - Graduated return-to-work with symptom-contingent microbreaks and stretch breaks; education in body mechanics and postural strategies to prevent recurrence. - Psychosocial/educational: - Pain education and CBT-based coping strategies as indicated (MTUS Chronic Pain CPG) when psychosocial barriers or delayed recovery are present. II. Right trapezius/shoulder girdle and right scapula (right trapezius strain; right scapulargia) - Medications: - As above: NSAIDs/acetaminophen/topical NSAID short-term; avoid opioids. - Rehabilitation: - Physical therapy focused on scapular stabilization, rotator cuff/periscapular strengthening, postural retraining, and activity modification; manual therapy adjunct short-term only; transition to HEP. Frequency/duration similar to neck recommendations, with continuation based on objective gains. - Acupuncture or myofascial release may be used as adjunctive, time-limited modalities when prior benefit is documented, with continuation only if measurable functional improvement occurs. - Procedures: - Trigger point injections (local anesthetic) for persistent, documented trapezial/scapular myofascial trigger points not responsive to conservative care, with limits per MTUS/ODG and with demonstrated functional benefit. - If clinical impingement syndrome develops/persists despite rehab, a single

image-guided subacromial corticosteroid injection may be considered; repeat injections only with clear functional improvement and within guideline limits. - Diagnostics: - If new/persistent mechanical shoulder symptoms suggest rotator cuff or labral pathology after an adequate trial of conservative care, consider diagnostic ultrasound or MRI per guideline criteria. - Work/ergonomics and prevention: - Task modification to minimize repetitive overhead work and sustained reach; microbreaks and stretch breaks; progressive strengthening to reduce recurrence risk. III. Right shoulder joint (non-specific pain; x-rays normal) - Management overlaps with section II: - Active rehabilitation (rotator cuff/scapular strengthening, postural training), activity modification, and HEP. - Consider subacromial corticosteroid injection only if persistent evidence-based impingement after failed rehab. - Advanced imaging only if persistent deficits/mechanical signs after guideline-concordant conservative care. - Acupuncture and manual therapy may be used as time-limited adjuncts with documented gains. IV. Right elbow (injury site noted in claim) - If clinical diagnosis is elbow tendinopathy/strain: - Activity modification; counterforce strap and/or wrist splint as indicated; eccentric forearm extensor strengthening via PT; stretching; topical NSAIDs preferred; brief oral NSAIDs as needed. - Injection therapy: corticosteroid injection offers short-term relief but is generally not favored for long-term outcomes in tendinopathy; if used, reserve for refractory cases with informed risk–benefit and limit frequency. PRP/biologics only in chronic refractory cases that meet ODG criteria and after UR. - If neuropathic symptoms localize to the ulnar nerve at the elbow (not supported on 08/28/2024 EMG/NCS), follow nerve entrapment guidelines (activity modification, night elbow extension splinting; therapy; limited injections; surgery only if severe or refractory with confirmatory studies). - Imaging (ultrasound/MRI) only if persistent dysfunction after guideline-concordant conservative care or if structural concern arises. V. Right wrist/hand (mild right carpal tunnel syndrome on 08/28/2024 EMG/NCS; address if within claim scope) - Conservative management per MTUS/ODG for mild CTS: - Neutral-position wrist splinting at night; activity modification and ergonomic adjustments for keyboard/mouse and repetitive hand tasks. - Short trial of hand therapy for nerve/tendon gliding, postural/ergonomic training, and HEP. - Consider a single ultrasound-guided local corticosteroid injection for persistent symptoms after initial conservative care; repeat only with documented benefit and within guideline limits. - Surgical release considered only after failure of an adequate course (generally 6–12 weeks) of conservative care or if electrodiagnostic criteria/severity and clinical findings warrant earlier intervention per guidelines. - Avoid routine repeat EMG unless clinical change or preoperative planning. Cross-cutting MTUS/ODG principles for all body parts - Emphasize active, function-focused care; use passive modalities sparingly and time-limited. - Continue any modality only with documented objective functional improvement (e.g., pain reduction, ROM, strength, work tolerance, validated scales) consistent with guideline criteria; otherwise, taper/stop. - Avoid long-term opioid therapy; if used at all, restrict to very short duration for acute severe pain with clear exit plan. - Education, HEP adherence, postural/ergonomic correction, and aerobic conditioning are central to preventing recurrence. - Advanced imaging and interventional procedures require guideline indications and should be preceded by adequate conservative care unless red flags are present. Records relied upon (examples) - 06/09/2023 cervical x-ray: mild disc space narrowing at C6–C7 and C7–T1. - 06/26/2024 cervical x-ray: straightening of lordosis; minimal degenerative changes, most at C6–C7 and C7–T1. - 08/28/2024 EMG/NCS: mild right carpal tunnel syndrome; chronic right C8 radiculopathy without ongoing denervation. - Diagnoses documented: neck muscle strain; right trapezius strain; right scapulargia; right shoulder pain. This plan conforms to MTUS (ACOEM-based) and ODG/MCG guidance for cervical strain/radiculopathy, shoulder/scapular myofascial disorders and nonspecific shoulder pain, elbow tendinopathy/strain, and mild carpal tunnel syndrome. Utilization should be governed by documented functional gains and standard MTUS/ODG criteria, with UR/IMR as applicable.